

Medical Knowledge Repository and Operational Network

An ecosystem for the medical asset



Why

medical operations and training are fragmented across multiple networks and servers that rely on consistent connectivity. We historically preference inaccessible desktop devices or have no true digital, mobile implementation that protects both patient and medic information.

Inconsistency costs us.

- 1) We have a severely limited operational picture with unclear hierarchy.
- 2) We lack access to familiar, consolidated digital tools
- 3) We spend time and bandwidth trying to appease a non-existent architecture



Objectives

Deliver an open-source, device-agnostic software that prioritizes a unit-oriented, offline-first mentality mindful of security and integration into legacy systems while avoiding third party vendor lock-in.

You own your data, and you have it when you need it most in a near-peer fight.

**EXPEDITED
TRIAGE AND
DOCUMENTATION**

**SECURE
MESSAGING AND
ADMIN TASKS**

**NAVIGATION AND
EVACUATION
MANAGEMENT**

**MEDICAL AND
TACTICAL
REFERENCES**

**TRAINING AND
EVALUATION
CONSOLIDATION**

**PROPERTY AND
EQUIPMENT
MANAGEMENT**



Measurables

EXPEDITED DOCUMENTATION

Sample: 634 encounters, 18 medics, 9 month timeframe.

Measure: complete ADTMC triage, screening, and documentation in less than ~5 minutes (SD 0.3 minutes). Medics reached a disposition in less than ~120 seconds (SD 0.4 minutes)

TRAINING VALIDATION

Sample: 18 medics, 6 month timeframe.

Measure: 370 NCO-led STP validations demonstrating increased competency with medical tasks and ADTMC algorithms.

COMMUNICATIONS

Sample: 6 separate training environments in limited connectivity across 12 months.

Measure: Use of On-Call functionality reduced man-hours by ~400. Use of discoverable providers and increased telehealth capabilities led to the improved evacuation of 7 Soldiers. **Without sacrificing medic personal contact information.**

PROPERTY MANAGEMENT

Sample: 24 medics across three units FY25,

Measure: CVIII usage and accountability reporting reduced man-hours by ~200, reduced budget requirements by ~\$3,200.00 USD.



Current Usage

Based on **self-reported** UIC. Current usage is reported by echelon. This is not demonstrative of entire echelon usage.

Usage data is elective opt-in and compliant with primary use, consumer privacy laws.

Any collected data is encrypted on device first. If stored in a server it is done so via hashed fields and RLS.

Guest users can opt to self-report usage data to the developer. No guest user information is collected by design even if it limits ability to report usage.

III CORPS - 154 users across

0061 MD

1 AD

1 CD

1 ID

75 FA

XVIII ABN - 44 users across

10 MTN

3 ID

82 ABN

I CORPS - 55 users across

11 ABN

4 ID

7 ID

USARNG - 52 users across

KY 149 MEB

CO 1-157 IN

FL 53 IN

V CORPS - 10 users across

41 FA

USMC - 10 users across

2ND BN



Roadmap



Phase 1

Consolidated documentation, evacuation, communication, training, property, and operations.



Phase 2 FY25 Q4

Expansion of training knowledge base to incorporate deployed medicine content consolidated with updated ICTLs



Phase 3 FY26 Q1

Expansion of property module to incorporate requisitions. Begin integration with legacy systems (GCSS-A, MODS, ATIS)



Phase 4 FY26 Q4 - FY27 Q4

Transition from PWA to app store (iOS, android, Windows). Transition to on-prem server. Integration with remaining legacy systems (MAVEN)



Phase 5 FY27 Q4

BAA. Integration into MHS. Parallel structure for non-medical units.



Open-Sourced. Always Accessible

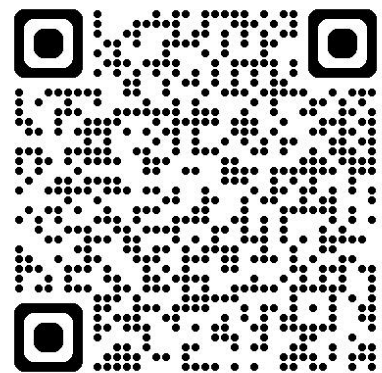
Available on all devices (across NIPR / DHA / personal). Entirely free to operate or implement into workflows, or contribute to the code base.

Repo:

<https://github.com/cconner2023/test2>

Web Application:

<https://cconner2023.github.io/test2>

**Desktop (edge/chromium):**

save and share -> install as app

iOS (safari/chromium)

share -> add to home screen

Android (chromium)

Options -> install as app

